

**Request for Information  
Virtual Emergent and Urgent Care Network  
VA-26-00045329  
February 27, 2026**

## **1. INTRODUCTION**

**THIS IS A REQUEST FOR INFORMATION (RFI) ONLY.** This is a Request for Information (RFI) only. Do not submit a quote. This RFI is for planning purposes only and shall not be considered an Invitation for Bid, Request for Quotation, or a Request for Proposal. Additionally, there is no obligation on the part of the Government to acquire any products or services described in this RFI. Your response to this RFI will be treated only as information for the Government to consider. You will not be entitled to payment for direct or indirect costs that you incur in responding to this RFI. This request does not constitute a solicitation for quotes or the authority to enter negotiations to award a task order/contract. No funds have been authorized, appropriated, or received for this effort. The information provided may be used by the Department of Veterans Affairs (VA) in developing its acquisition strategy and Performance Work Statement/Product Description. Any information submitted by respondents to this RFI is strictly voluntary; however, any information received shall become the property of the Government and will not be returned to the respondent. Interested parties are responsible for adequately marking proprietary, restricted or competition sensitive information contained in their response. The Government does not intend to pay for the information submitted in response to this RFI. Product information, brochures, part numbers, and/or other description information may be included with the submission.

Be advised that set-aside decisions may be made based on the information provided in response to this RFI. Responses should be as complete and informative as possible.

## **2. STRATEGIC VISION**

The Veterans Health Administration (VHA) envisions a next generation national virtual care model that offers up to 4 million Veterans seeking acute, unscheduled, urgent or emergent care each year. The model will aid in determining which Veterans can be managed safely and effectively in a virtual setting and which Veterans need in-person care. This will expand options for Veterans who are located far from health care facilities or prefer to receive care at home. Additionally, this capability will help reduce the number of low-value, in-person emergency care visits, which increase emergency room crowding and wait times for higher-acuity patients and drive increased health care costs.

While VHA ultimately seeks a comprehensive solution, it is open to a modular and phased approach to development and implementation. VHA welcomes submissions from vendors who can meet most of its requirements and have a clear roadmap towards delivering a complete solution. The ideal solution encompasses the entire Veteran journey, from first contacting the VHA, through intake, accurate triage, virtual treatment, and diversion to in-person care if necessary. It includes capturing patient history, conducting virtual exams, and medical assessments by credentialed providers, as well as charting, imaging and laboratory order placement, prescription placement, discharge,

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and after-visit follow up. The solution should also support warm hand-offs, facilitate collaboration and communication among various clinical staff during a patient encounter, and provide care coordination support with the Veterans' primary care physician and the Patient Aligned Care Team. Solutions may leverage advanced technology, artificial intelligence, software platforms, tailored clinical protocols, a distributed provider network (hub and spoke), home-based monitoring, communication tools, and other innovative features.

The Virtual Emergent and Urgent Care Network will work across the VHA enterprise, maintaining continuity with our existing infrastructure (including both Computerized Patient Record System (CPRS) and the Oracle Federal Electronic Health Records (EHR). Veterans will experience outstanding on-demand VHA care at the right level of clinical acuity in the best location to suit their needs.

### **3. RFI RESPONSE INSTRUCTIONS**

If you can provide the necessary supplies or services, please respond to this RFI. There is a page limitation for this RFI of 15 pages. The Government will not review any other information or attachments included, that are in excess of the 15-page limit. **NO MARKETING MATERIALS ARE ALLOWED AS PART OF THIS RFI.** Generic capability statements will not be accepted or reviewed. Your response must address capabilities specific to the services required in *Attachment 1 - Virtual Emergent and Urgent Care Network Requirements* and must include the following:

- a. Interested Vendors shall at a minimum provide the following information in the initial paragraph of the submission:
  - Name of Company
  - SAM Unique Entity ID
  - Commercial and Government Entity Code
  - Address
  - Point of Contact Name
  - Phone Number
  - Fax Number
  - Email address
  - Company Business Size and Status
  - NAICS code(s)
  - Socioeconomic data
  - Existing Contractual Vehicles the contractor holds for providing these services (Government-Wide Acquisition Contract (GWAC), Federal Supply Schedule, other agency GWACs or Multiple Award Contract, etc.)
  - For Service-Disabled Veteran Owned Small Business (SDVOSB) / Veteran Owned Small Business (VOSB), proof of verification in the U.S Small Business Administration Veteran Small Business Certification.

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- b. Provide a response to meet the requirements contained within *Attachment 1 - Virtual Emergent and Urgent Care Network Requirements*. Respondents shall clearly indicate how their solution does or does not meet each of the requirements in the attachment. For requirements not currently met by the respondent's solution, please include information regarding plan, estimated cost, and timeline to implement the requirement.

Note: VA is requesting information on two distinct areas: 1) provider-facing software platform; and 2) staffing models and delivery structure. Responses may address one or both areas.

- c. Corporate experience or expertise in performing these services and specific examples or references. Specific examples or references provided must include the agency, point of contact, dollar value, and contract number.
- d. Has *Attachment 1 - Virtual Emergent and Urgent Care Network Requirements* provided sufficient detail to describe the technical requirements that encompass the services to be performed under this effort.

\_\_\_\_\_ YES \_\_\_\_\_ NO (if NO, please answer question e.)

- e. If "NO", please provide your technical comments/recommendations on elements of the *Attachment 1 - Virtual Emergent and Urgent Care Network Requirements* that may contribute to a more accurate proposal submission and efficient, cost-effective effort.

#### **4. RESPONSES**

Note: Provide a capability package that is clear, concise, and complete. VA is under no obligation to provide feedback to the company, or to contact the company for clarification of any information submitted in response to this RFI.

Responses are due no later than 3:00 p.m. ET, March 27, 2026 via email to Contracting Officer John Vardouniotis at [ioannis.vardouniotis@va.gov](mailto:ioannis.vardouniotis@va.gov) and Contract Specialist Elena Juliano at [elena.juliano@va.gov](mailto:elena.juliano@va.gov). Please note "Virtual Emergent and Urgent Care Network Requirements" in the subject line of your response. Mark your response as "Proprietary Information" if the information is considered business sensitive. The email file size shall not exceed 25 MB.

OFFERORS ARE ADVISED THAT THIS REQUIREMENT MAY BE DELAYED, CANCELED, OR REVISED AT ANY TIME.